CLINICAL NOTE · RED EYE, INJURY, FLASHES & FLOATERS, SUDDEN SYMPTOMS

Problem-Focused & Urgent Eye Visit

Focused note for acute and medical visits: triage red flags, targeted history and examination, treatment, return precautions and referral urgency.

Completed by: Clinician | Use as: Note template | Time: 10–20 min

Patient name	Date of birth	Patient ID / MRN	Date of service	Clinician

01 Triage

Act first
Chemical injury: irrigate immediately (≥ 1–2 L saline / lactated Ringer's) until pH 7.0–7.5 BEFORE further history. Sweep fornices and recheck pH after 5–10 minutes.

Red flags present (tick all)

- ☐ Sudden painless vision loss
- ☐ Curtain / shadow in vision
- ☐ Shower of new floaters or flashes
- ☐ Penetrating injury / globe rupture risk
- ☐ Severe pain with halos, nausea (angle closure)
- ☐ Contact lens wearer with painful red eye
- ☐ Photophobia + ciliary flush (anterior uveitis)
- ☐ New diplopia, ptosis or anisocoria
- ☐ Age > 50 with jaw claudication / scalp tenderness (GCA)
- ☐ Headache with optic disc swelling
- ☐ Post-operative or post-injection pain / vision drop
- ☐ Neonate with conjunctivitis

Triage category

○ Emergency — now ○ Same day ○ Within 24–48 h ○ Routine

Time of onset / injury

02 History of present illness

Chief complaint / reason for visit (patient's own words)

Location / eye	Onset & duration	Severity (0–10)	Timing / frequency

Quality / character	Context / triggers	Modifying factors	Associated signs & symptoms

Mechanism (if injury)

- ☐ Blunt
- ☐ Sharp / penetrating
- ☐ Metal on metal / grinding
- ☐ Chemical (acid / alkali)
- ☐ Thermal / UV / welding
- ☐ Fingernail / plant
- ☐ Other: _____

Eye protection worn

○ Yes ○ No ○ N/A

Contact lens wear

○ No ○ Yes — in at onset ○ Yes — removed

Treatment already tried

Relevant ocular / medical history, medications, allergies

Tetanus status (if injury) / exposures / sick contacts

03 Examination

Visual acuity	Distance SC	Distance CC	Pinhole	Near SC	Near CC
OD					
OS					
OU					

Pupils	Size dim (mm)	Size bright (mm)	Shape	Direct	Consensual	RAPD
OD	mm	mm				○ None ○ 1–4+: _____
OS	mm	mm				○ None ○ 1–4+: _____

Tonometry	IOP	Method	Time	CCT (pachymetry)	Notes
OD	mmHg			µm	
OS	mmHg			µm	

Motility
○ Full ○ Restricted / painful: _____

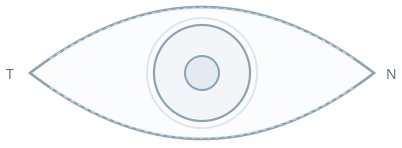
Confrontation fields
○ Full ○ Defect: _____

Pain on eye movement
○ No ○ Yes

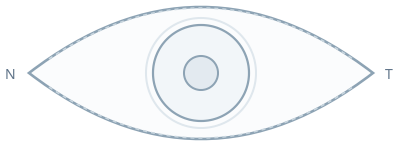
Structure	OD	OS
Lids / lashes (evert for FB)	☐ WNL	☐ WNL
Conjunctiva (injection pattern)	☐ WNL	☐ WNL
Discharge	☐ WNL	☐ WNL
Cornea	☐ WNL	☐ WNL
Fluorescein staining	☐ WNL	☐ WNL
Seidel test	☐ WNL	☐ WNL
Anterior chamber (cell / flare / hypopyon / hyphaema)	☐ WNL	☐ WNL
Iris / pupil	☐ WNL	☐ WNL
Lens	☐ WNL	☐ WNL
Pre-auricular nodes	☐ WNL	☐ WNL

Posterior segment	OD	OS
Anterior vitreous (Shafer's sign / pigment)	☐ WNL	☐ WNL
Vitreous (PVD / haemorrhage)	☐ WNL	☐ WNL
Optic nerve	☐ WNL	☐ WNL
Macula	☐ WNL	☐ WNL
Periphery (scleral depression)	☐ WNL	☐ WNL

Lesion / FB location — OD



Lesion / FB location — OS



04 Procedures performed

- ☐ 65222 Corneal foreign body removal (slit lamp)
- ☐ 65210 Conjunctival FB removal, embedded
- ☐ 92071 Bandage contact lens for ocular surface disease
- ☐ 65430 Corneal scraping for diagnosis / culture
- ☐ 65205 Conjunctival FB removal, superficial
- ☐ 67820 Epilation (trichiasis), forceps
- ☐ 68761 Punctal plug, each
- ☐ 68020 Incision of conjunctiva / drainage

Anaesthetic / instrument	Rust ring removed?	Cultures sent / lab	Complications

05 Assessment & plan

- ☐ S05.0- Corneal abrasion without FB
- ☐ B30.9 Viral conjunctivitis
- ☐ H20.0- Acute / subacute iridocyclitis
- ☐ H11.3- Subconjunctival haemorrhage
- ☐ T15.0- Foreign body in cornea
- ☐ H10.1- Acute atopic conjunctivitis
- ☐ H43.81- Vitreous degeneration (PVD)
- ☐ H00.0- Hordeolum
- ☐ H10.3- Acute conjunctivitis, unspecified
- ☐ H16.0- Corneal ulcer
- ☐ H33.3- Retinal break without detachment
- ☐ H15.1- Episcleritis

> Codes ending in "-" need laterality / encounter characters (e.g. S05.01XA = right eye, initial encounter).

#	Diagnosis / assessment	ICD-10	Eye	Status / severity	Plan
1					
2					
3					

Medication prescribed	Dose / strength	Eye / route	Frequency	Start date	Notes / adherence

Return precautions given

- ☐ Worsening pain
- ☐ Decreasing vision
- ☐ Increasing redness / discharge
- ☐ New flashes / floaters / curtain
- ☐ No contact lens wear until cleared
- ☐ Work / school exclusion advised

Referral to / urgency / time called

Work / school note issued

Return to clinic

☐ 1 day ☐ 1 week ☐ 2–4 weeks ☐ 3 months ☐ 6 months ☐ 12 months ☐ PRN

Specific RTC date / interval

06 Attestation

Clinician signature

×

Clinician name & credentials

Date / time signed

License / registration no.

NPI / provider no.

Technician / scribe

Co-signing clinician (if required)